

MANAGEMENT OF FALLS AND FRACTURES



What you do at the time of a fall is really important. Safe moving and handling and prompt, appropriate care and attention can greatly improve a resident's chance of making a full recovery. The immediate care of a resident, following a fall, should include safety at the scene and addressing any injuries sustained. An inappropriate response can delay the diagnosis and treatment of serious injuries. The responses made should be in keeping with an individual's Palliative Care Summary (PCS), Anticipatory Care Plan (ACP) including Verification of Expected Death and Do Not Attempt Cardio Pulmonary Resuscitation (DNACPR) decision recording and guidance.

Steps to take:

- Ensure safety at the scene.
- Assess for and attend to obvious injury.
- Ask for additional help as required.
- Safely move the resident from the floor (where appropriate).
- Report and comprehensively record the fall and the consequences of the fall, including completing:
 - An accident/incident report form required by the organization (old people's home)
 - A post falls report

– The falls diary/update the database

- Analyze the causes and compile and implement an action plan to prevent further falls

1. When a resident has fallen or has been found on the floor

- Check first for ongoing hazards or dangers.
- Check if the resident is responsive.
- If responsive, provide reassurance and comfort to the resident who has fallen.
- Summon help from other members of staff.
- If unresponsive, check the resident's airways, breathing and circulation.
- Do not move the resident before checking for pain, loss of sensation (feeling), loss of movement in arms and/or legs, and observe for swelling, visible injury and deformity.
- Check for nausea, confusion, drowsiness, delirium and agitation.
- Commence routine observations such as resident's temperature, pulse, BP and respirations as appropriate.
- Call the emergency GP, or an ambulance if appropriate.
- If the decision is taken to move an injured resident from the floor, ensure staff have the expertise and equipment to do so safely, and that moving and handling guidelines are followed.
- Complete accident/post falls report form, record in the care plan and inform next of kin as agreed.

2. If unconscious or unresponsive

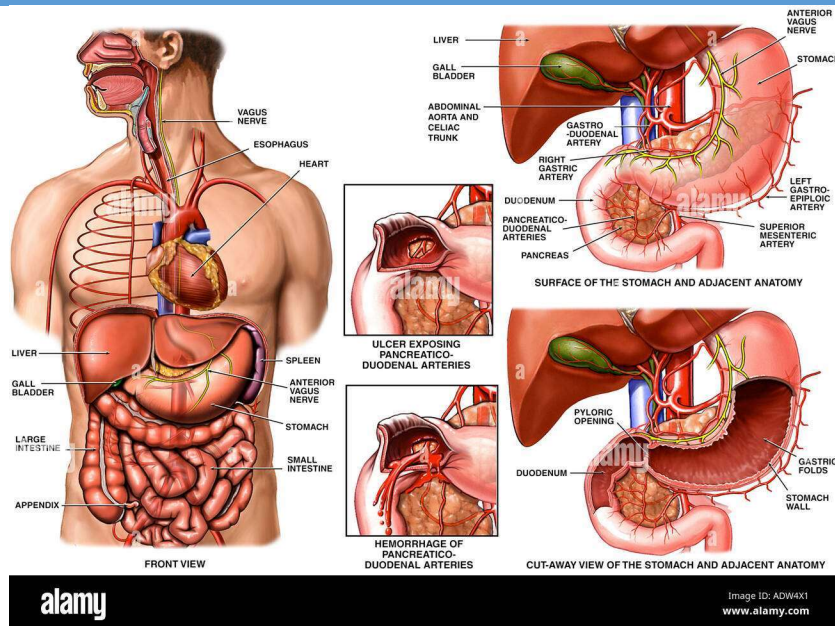
- The response should be in keeping with the recorded Anticipatory Care Plan (ACP) for the individual including their Verification of Expected Death, Do Not Resuscitate (CPR) and care home policy and their Palliative Care Summary (PCS).
- If indicated necessary and appropriate undertake Cardio Pulmonary Resuscitation (CPR).
- If consciousness is transient and resident appears to recover back to full functional level contact GP or Ambulance.

- If ongoing impaired conscious level, worsening cognition or new focal neurological signs phone ambulance and inform ambulance staff of the ACP or PCS.
- Record resident's temperature, pulse, BP and respirations.
- Complete accident/post falls report form, record in the care plan and inform next of kin as agreed.

3. If injury or change in health suspected

- If head trauma, spinal damage or lower limb fracture is suspected make the resident comfortable on the floor. **DO NOT MOVE THE RESIDENT.**
- If there is a suspected head or spinal injury begin neurological observations ie Glasgow Coma Score (GCS), that is eye-opening, motor, and verbal responses •
- If there is a suspected upper limb fracture, immobilise the limb (if trained and confident to do so) and return the person to bed/chair.
- If not trained and confident to immobilize the limb make the person comfortable on the floor.
- Ascertain if the resident has a completed ACP or PCS.
- According to local agreement call emergency GP or an ambulance as appropriate and inform emergency staff of the ACP or PCS.
- Complete accident/post falls report form, record in the care plan and inform next of kin as agreed.

HEMORRHAGE



Hemorrhaging is excessive bleeding that is very difficult to stop. It is rare but can happen with certain medical conditions. Stomach or bowel tumors, or tumors that are weakening arteries may cause this bleeding. It can be from an external injury (such as a stab wound or a compound fracture) or internal injury (such as bleeding ulcer or trauma to the chest or stomach). Internal hemorrhage may show up as skin discoloration or blood in the urine, stool or vomit. The person will probably feel pain and tenderness in the affected area.

Your Main Objective is to control the bleeding and get help fast:

- always use standard precautions anytime you come into contact with blood. Remember to keep an extra pair of latex gloves with you at all times.
- Check for consciousness, respiratory arrest (no breathing), or cardiac arrest (heart stops breathing). Stopping bleeding won't help if the person isn't breathing.
- Call for emergency services.

Basic tips

- while you waiting for help, have the person lie down.
- Apply direct pressure.

- Cover the wound with a sterilized gauze or pad. This will help the blood to clot and stop the bleeding.
- If blood soaks through the gauze, don't remove it. Instead, place another gauze on top and continue to apply pressure to the wound for five or more minutes.
- Elevate the injury. If possible, position the wounded part of the body above the level of the heart while you apply direct pressure.
- After five minutes, if direct pressure and elevation hasn't worked, you'll need to do more to try to stop the bleeding. Large arteries that are found close to the skin's surface supply blood to the head, arms and legs. The bleeding may stop if you apply pressure to the major artery supplying blood to the wound. You'd do this by pressing your middle three fingers over the artery's pressure point. If using the pressure point on a leg, you may need to use the heel of your hand instead of your fingers. Be sure not to cut off circulation completely.

